

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Randomized, Phase ? Study of Rilvegostomig in Combination With Fluoropyrimidine and Trastuzumab Deruxtecan Versus Trastuzumab, Chemotherapy, and Pembrolizumab for the First Line Treatment of HER2-positive Gastric Cancer (ARTEMIDE-Gastric01)
Short Title:	A Phase ? Study of Rilvegostomig in Combination With Fluoropyrimidine and Trastuzumab Deruxtecan as the First-line Treatment for HER2-positive Gastric Cancer
Protocol Number:	1059499775530863043
NCT Number:	NCT06764875
Sponsor Name:	AstraZeneca
Investigational Product:	cisplatin
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Progression free survival (PFS): PFS is defined as time from randomization until progression per RECIST v1.1, or death due to any cause. 2. Overall Survival (OS): OS is defined as time from randomization until the date of death due to any cause.
Secondary Obj.:	1. Objective Response Rate (ORR) 2. Duration of Response (DoR) 3. Proportion of all randomized participants alive and progression-free at 6 months (PFS6)

2.2 Background & Rationale

This is a Phase ?, randomized, open-label, Sponsor-blinded, 3-arm, global, multicenter study assessing the efficacy and safety of rilvegostomig in combination with fluoropyrimidine and T-DXd (Arm A) compared to trastuzumab, chemotherapy, and pembrolizumab (Arm B) in HER2-positive locally advanced or metastatic gastric or GEJ adenocarcinoma participants whose tumors express PD L1 CPS ? 1. Rilvegostomig in combination with trastuzumab and chemotherapy will be evaluated in a separate arm (Arm C) to assess the contribution of each component in the experimental arm.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	840

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

* HER2 positive for gastric cancer on a tumor biopsy.

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- * PD-L1 combined positive score (CPS) ≥ 1.
- * Provision of tumor tissue sample from recent biopsy adequate for HER2 and PD-L1 testing.
- * Previously untreated, unresectable, locally advanced or metastatic gastric or GEJ adenocarcinoma.
- * WHO or Eastern Cooperative Oncology Group performance status of 0 or 1.
- * Have measurable target disease assessed by the Investigator based on RECIST v1.1.
- * Have adequate organ and bone marrow function within 14 days before randomization.
- * LVEF ≥ 55% within 28 days before randomization.
- * Adequate treatment washout period before randomization.

4.2 Exclusion Criteria

- * Lack of physiological integrity of the upper gastrointestinal tract.
- * Known dihydropyrimidine dehydrogenase enzyme deficiency.
- * Contraindication to pembrolizumab or trastuzumab, contraindications to fluoropyrimidine (5-FU and capecitabine) or platinum (cisplatin and oxaliplatin) treatment as per local label.
- * History of another primary malignancy except for malignancy treated with curative intent with no known active disease within 3 years before the first dose of study intervention and of low potential risk for recurrence.
- * Persistent toxicities caused by previous anti-cancer therapy.
- * Spinal cord compression or brain metastases unless asymptomatic, treated and stable and not requiring corticosteroid or anticonvulsant may be included in the study if they have recovered from the acute toxic effect of radiotherapy.
- * Uncontrolled infection including tuberculosis and active hepatitis A infection.
- * Uncontrolled infection requiring intravenous (IV) antibiotics, anti-virals, or antifungals.
- * Recent receipt of live, attenuated vaccine.
- * Chronic/active HBV or HCV infection unless controlled.
- * Clinically significant cardiac or psychological conditions.
- * Active or prior documented autoimmune or inflammatory disorders requiring chronic treatment with steroids or other immunosuppressive treatment.
- * History of (non-infectious) ILD/pneumonitis, has current ILD/pneumonitis, or where suspected ILD/pneumonitis cannot be ruled out by imaging at screening.
- * Lung-specific intercurrent clinically significant illnesses.
- * Any active non-infectious skin disease requiring systemic treatment.
- * A pleural effusion, ascites or pericardial effusion that requires drainage, peritoneal shunt, or cell-free and concentrated ascites reinfusion therapy (CART).
- * History of any of the following: drug-induced severe cutaneous adverse reaction.
- * Any concurrent anti-cancer treatment with the exception of receptor activator of nuclear factor kappa-B ligand inhibitors.
- * Have had major surgical procedure recently (excluding placement of vascular access) or recent significant traumatic injury or an anticipated need for major surgery during the study.
- * Current or prior use of immunosuppressive medication within 14 days before study intervention.

11. Study Identifiers & Governance

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Registry Number: NCT06764875

Sponsor: AstraZeneca

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15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____